



You or your partner can enjoy the first hold of your baby while the surgical team completes the operation by delivering the placenta and stitching the incision.

Your recovery

Your nurses will encourage you to get out of bed that same day, and your obstetrician may recommend removing the urinary catheter after a few hours. This will allow you to feel more comfortable and decrease your risk of developing a bladder infection. After your Cesarean, you can eat and drink as you wish, although you may find you don't necessarily have the appetite you anticipated. The day after your surgery, your obstetrician will remove your bandage, if one was placed, to inspect the incision. As you move about more, you may find it easier to do so by using an abdominal binder. By the day after your Cesarean, you should be well enough to do most things for yourself, with help. Women usually go home on the second or third day after the Cesarean with NSAIDs and acetaminophen for pain relief.

Your next pregnancy

If you need to have a Cesarean section, your next pregnancy will be considered high risk. Although rare, you will be at increased risk in subsequent pregnancies for placenta accreta spectrum, which is a range of changes in the development of the placenta that can lead to it growing abnormally into the uterine wall, or even outside of the uterus. The risk of this happening increases with the number of previous Cesareans. This is why most obstetricians do not recommend having more than three or four Cesareans. Pregnancies with placenta accreta spectrum are likely to require a hysterectomy at the time of Cesarean delivery.

COMPLICATIONS

There are several common, but minor, problems associated with Cesareans. These include bleeding during the surgery, or a day or so later; the need for a blood transfusion; or getting a minor infection in the bladder or in the abdominal incision. Getting a major infection is far less